DPP — DEPARTMENTAL POLICY AND PROCEDURE

TITLE:	CARE OF NEWBORN WITH TRANSIENT TACHYPNEA
DEPARTMENT:	NEONATAL INTENSIVE CARE UNIT
APPLIES TO:	NICU PHYSICIANS AND NURSES

CODE AND VERSION NUMBER	
DPP-NICU-0013 (v4)	
LAST VERSION APPROVAL DATE:	28 / 05 / 2023
NUMBER OF PAGES:	5

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APPROVED DATE	EFFECTIVE DATE	TYPE OF REVISION	REVIEW DATE
09 / 02 / 2026	23 / 02 / 2026	TRIENNIAL	08 / 02 / 2029

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1. PURPOSE

1.1. To provide guidelines to physician on how to manage sick newborn with transient tachypnea.

1.2. To emphasize the nursing care to be rendered to patients with TTNB.

2. DEFINITION & ABBREVIATION

2.1. **TTNB (Transient Tachypnea of the Newborn)** is a self-limited disease of full-term or near term infants that is characterized by fast breathing that usually resolves over 24-72 hours.

2.2. **Tachypnea** – rapid breathing between 60 – 120 breaths per minute.

2.3. **Retraction** – is a sharp pulling in of the chest muscles during breathing.

2.4. **PROM** – Premature Rupture of Membranes

2.5. **OGT** – Oro-Gastric Tube Feeding

2.6. **VBG** – Venous Blood Gases

2.7. **ECG** – Electrocardiogram

2.8. **PPE** – Personal Protective Equipment

2.9. **NICU** –Neonatal Intensive Care Unit.

3. RESPONSIBILITY

3.1. **NICU Physicians (Consultant, Specialists and Residents)**

3.1.1. Responsible to assess and evaluate patients for transient tachypnea.

3.1.2. The physician should provide a written order of the procedure/management of TTNB to be done to the patient in the medical file.

3.2. **NICU Nurse (Head Nurse, Charge Nurse and Staff Nurse)**

3.2.1. Will assess and monitor respiratory rate and other vital signs of the patient.

3.2.2. Responsible in documenting the given procedure in the patient's medical file.

4. POLICY

4.1. All newborn patients who develop non-infectious acute respiratory disease must be carefully placed under observation in Nursery and will be subjected for admission to Neonatal Intensive Care Unit if indicated according to the newborn's clinical condition and physician's written order.

4.2. The physician should review the antenatal, delivery and postnatal history of the newborn's mother including key factors such as:

4.2.1. PROM

4.2.2. Cesarean delivery or rapid vaginal delivery

4.2.3. Maternal pyrexia

4.2.4. Maternal asthma and smoking

4.2.5. Maternal sedation

4.2.6. History of feeds

4.2.7. Known anomalies

4.3. Careful assessment must be given to infants with TTNB that comes out within the first few hours of life which present the following:

4.3.1. Clinical Presentation

4.3.1.1. Manifests in full term infants, large for gestational age babies

4.3.1.2. Tachypnea (60-120 per minute)

4.3.1.3. Presence of nasal flaring, grunting, retractions

4.3.1.4. Barrel chest (symmetric hyperinflation) for some infants

4.3.1.5. Cyanosis which appears to resolve with less than 40% oxygen

4.4. Blood investigations such as full blood count, septic screening and blood gases must be done.

4.5. Imaging studies such as chest X-ray, ECG and echocardiography (is suspecting congenital heart disease) must ordered by the physician.

4.6. Supplemental oxygen will be given depending on the patient's ability to maintain adequate arterial oxygen concentration.

4.7. Empiric antibiotics should be started until infection has been ruled out. Intravenous ampicillin and gentamicin are used.

4.8. Gavage feedings are given until the respiratory rate has decreased enough to allow oral feedings. OGT feedings should be on hold if respiratory rate is above 80 breaths per minute.

4.9. Intravenous fluids should be administered while on restriction of feedings.

4.10. Nursing care must be carefully rendered depending on the physician's written order and management of TTNB such as:

4.10.1. Continuous assessment and monitoring of the patient's respiratory rate and other vital signs.

4.10.2. Maintaining a neutral thermal environment with minimal stimulation to the sick newborn.

4.10.3. Intravenous fluids must be properly checked to avoid fluid overload that can worsen respiratory manifestations.

5. PROCEDURE

5.1. The physician and the nurse will assess the patient for transient tachypnea of the newborn.

5.2. The physician will provide the written order of the procedure to be done to manage or treat

TTNB.

5.3. The nurse will verify and carry out doctor's order.

5.4. Identify the patient correctly.

5.5. Do hand hygiene and wear appropriate PPE according to the procedure to be done.

5.6. The nurse will proceed in performing the procedure to manage TTNB according to physician's order.

5.6.1. Observation and monitoring of vital signs

5.6.2. Oxygen therapy

5.6.3. Pulse oximetry monitoring

5.6.4. Suctioning

5.6.5. Antibiotic therapy

5.7. Perform continuous re-assessment and follow-up blood culture and CRP report of the sick newborn.

5.8. The physician will change the line of antibiotics, if blood culture and CRP are positive.

5.9. The physician and nurse will continue to re-evaluate the newborn's condition.

5.10. If the newborn remains vitally and clinically stable, stop antibiotic therapy, if blood culture and CRP reports are negative, according to physician's written order.

5.11. The nurse will provide the appropriate nursing care for the newborn patient according to physician's order and management.

5.12. Document all the procedures done in the patient's medical file.

6. MEASUREMENT

6.1. 100% of the NICU staff is knowledgeable and adhere to this policy and procedures.

7. REFERENCES /CROSS- REFERENCES

7.1. PMSH-DPP-NICU-0013(v3): Care of Newborn with Transient Tachypnea, 11/06/2023, p. 1-4

7.2. CBAHI Standards 3rd edition, 01/01/2016, NICU.10, NICU.12 p.126-127

7.3. Newborn Services Guidelines and Protocols in the Royal College of Pediatric website

7.4. National Institute for Health & Clinical Excellence UK

8. ATTACHMENT

8.1. None

9. REVISION CLAUSE

9.1. Changes to the hospital policy and procedure template based from the Al-Baha Health Cluster logo, font face and design.